



TIRZEPATIDE RESEARCH HUB · RESEARCH USE ONLY

# BURN THE FAT. GUARD THE HEART.

FOR MEN & WOMEN. Burn fat and keep your heart healthy.

Get G2-T 30 mg at BioLinx

Read the guide first

Also shown to help with: diabetes · insulin resistance · appetite control & binge eating · pre-diabetes · pcos

Once per week · at night · Difficulty: beginner

## WHAT'S INSIDE

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## RESEARCH SHOWED THAT...

**20%** body weight  
lost in under 18 months.

NEJM, 2025

People on tirzepatide lost  
nearly twice the weight of  
people on Ozempic.

NEJM, 2021



STEP 1 OF 4 · WHAT TO BUY

# SHOPPING LIST

Easy-to-follow shopping list to safely practice Tirzepatide. Match your commitment level — don't skip anything.



PEPTIDE · ONE ORDER

## G2-T 30 MG

Burn fat and keep your heart healthy.  
Common starting dose: **1.25 mg**, once per week, at night.

**Get G2-T 30 mg at BioLinX**

[biolinxlabs.com/products/g2-t-30-mg](https://biolinxlabs.com/products/g2-t-30-mg)

HOW MANY VIALS TO BUY · BEST VALUE BY GOAL

| GOAL                        | VIAL       | QTY | WHERE                                                                                                                 |
|-----------------------------|------------|-----|-----------------------------------------------------------------------------------------------------------------------|
| 1 MONTH                     | 10 mg vial | 1×  | <b><u>See it</u></b><br><a href="https://biolinxlabs.com/products/g2-t-10-mg">biolinxlabs.com/products/g2-t-10-mg</a> |
| 4 MONTHS <b>RECOMMENDED</b> | 30 mg vial | 4×  | <b><u>See it</u></b><br><a href="https://biolinxlabs.com/products/g2-t-30-mg">biolinxlabs.com/products/g2-t-30-mg</a> |
| FULL CYCLE                  | 30 mg vial | 7×  | <b><u>See it</u></b><br><a href="https://biolinxlabs.com/products/g2-t-30-mg">biolinxlabs.com/products/g2-t-30-mg</a> |

MATERIALS ≈ \$41 ON TOP — FULL LIST ON THE NEXT PAGE



## PRODUCTS ARRIVED?

# QUICK SUMMARY OF YOUR LAB ITEMS:

TWO VIALS IN THE BOX • G2-T 30 MG + BACTERIOSTATIC WATER 10 ML

VIAL 1 **G2-T 30 mg.** Arrives as a powder. Don't touch it until you've read page 4.

VIAL 2 **Bacteriostatic water — 10 mL.** This is what turns the powder into a liquid. Sterility and pH matter — don't substitute.

## GROCERY LIST • ≈ \$41

| ITEM                                | SPEC                                                                             | QTY       | WHERE                                                                                                                                          |
|-------------------------------------|----------------------------------------------------------------------------------|-----------|------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>INSULIN SYRINGES</b> <b>BUY</b>  | 1cc U100, 30G, 5/16"                                                             | 50–100 ct | Any pharmacy                                                                                                                                   |
| <b>ALCOHOL PREP PADS</b> <b>BUY</b> | Any brand, 70% isopropyl                                                         | 50–100 ct | Any pharmacy                                                                                                                                   |
| <b>SHARPS CONTAINER</b> <b>BUY</b>  | 1 quart, for needle disposal                                                     | 1×        | Any pharmacy                                                                                                                                   |
| <b>BAC WATER</b> <b>BUY</b>         | Bacteriostatic water,<br>10 mL — sterility and<br>pH matter, don't<br>substitute | 1 vial    | <b>See it</b><br><a href="https://biolinxlabs.com/products/bacteriostatic-water-10-ml">biolinxlabs.com/products/bacteriostatic-water-10-ml</a> |

Any brand works for syringes, pads and sharps — but not BAC water. Use 2 fresh syringes and 2 fresh pads every session: one to mix, one to inject.

## HOW TO STORE IT

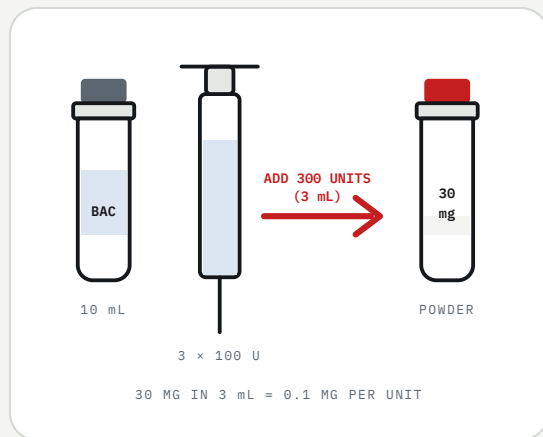
|               |                                                         |
|---------------|---------------------------------------------------------|
| POWDER        | Freezer — lasts years                                   |
| RECONSTITUTED | Fridge — about 28 days. Write the mix date on the vial. |
| WATER         | Room temperature — check the expiration date            |



STEP 2 OF 4 · RECONSTITUTION – FROM POWDER TO LIQUID

# LET'S “FORM” YOUR PEPTIDE!

“Form” means “reconstitute”. Your product arrives as a powder — you need to liquify it before “you begin your research.” 100 units = one full syringe. **10 mg vial → add 100 units (1 mL) · 30 mg vial → add 300 units (3 mL).** Every dose in this guide assumes exactly that ratio, so measure it — don't eyeball it.



## YOUR SIMPLIFIED UNITS

| DOSE | FILL TO |
|------|---------|
|------|---------|

|         |            |
|---------|------------|
| 1.25 mg | 12.5 units |
|---------|------------|

|        |          |
|--------|----------|
| 2.5 mg | 25 units |
|--------|----------|

|      |          |
|------|----------|
| 5 mg | 50 units |
|------|----------|

|        |          |
|--------|----------|
| 7.5 mg | 75 units |
|--------|----------|

|       |           |
|-------|-----------|
| 10 mg | 100 units |
|-------|-----------|

|         |                      |
|---------|----------------------|
| 12.5 mg | 125 units – 100 + 25 |
|---------|----------------------|

|       |                      |
|-------|----------------------|
| 15 mg | 150 units – 100 + 50 |
|-------|----------------------|

1

### Remove the plastic caps & wipe both rubber vial tops

Fresh alcohol pad on the G2-T vial and the bac water vial.

2

### Draw 300 units of bac water

That's 3 full pulls on a 100-unit insulin syringe (3 mL total).

3

### Inject it slowly down the side of the 30 mg vial

Let it run down the glass. Don't blast the powder.

4

### Swirl gently. Never shake. Place it in the fridge!

Slow and gentle wins this one — shaking wastes peptide. Discard that syringe into the sharps container. Let it rest until completely clear (36–46 °F); use within ~28 days; never freeze it.



STEP 3 OF 4 · INJECT

# WHERE TO INJECT

Time to inject · clean, pinch, poke, done. Once per week, at night.

1

## Take G2-T out of the refrigerator and extract your dose

Your first dose is **1.25 mg = 12.5 units** on a 100-unit syringe. Wipe the vial top with a fresh alcohol pad, let it air-dry a few seconds, then draw your units.

2

## Pinch a fold of skin on the stomach + use an alcohol wipe

Two inches away from the belly button. Wipe with a fresh alcohol pad, let it air-dry, and don't touch that spot again before injecting. Rotate sites.

3

## Needle in at the base of the fold

Push the plunger slowly and steadily. Hold for a few seconds, then withdraw.

4

## That's it — you just did your first shot!

Drop the needle straight into the sharps bin. Vial goes back in the fridge.



STOMACH · PINCH A FOLD · NEEDLE AT THE BASE

### WATCH FIRST

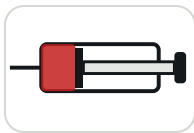
Mixing and injecting from start to finish, on video: [professorpeptides.com](https://professorpeptides.com). Use 2 fresh syringes and 2 fresh pads every session — one to mix, one to inject.



STEP 4 OF 4 · DOSING & TIMING SCHEDULE

# TIRZEPATIDE SCHEDULE (PROTOCOL)

“Schedule” means “protocol”. Everything below assumes the 30 mg vial was reconstituted with exactly 300 units (3 mL), read off a standard 100-unit insulin syringe.



## FIRST DOSE

How much, how often & when?

WHEN

**At night.** Once per week.

HOW MUCH

**1.25 mg** to start.

Dose steps up week by week with no breaks — this is called **titration**. Full schedule on page 9.

UNITS

Extract **12.5 units** on a 100-unit insulin syringe.

12.5 units = 1.25 mg. Exact variables: 100-unit syringe · 30 mg vial reconstituted with 300 units · 0.1 mg per unit.

HOW OFTEN

**Once per week.** Hold each step for its full block before moving up.

RUN LENGTH

**16-week minimum cycle.**

**Don't rush it — start at the lowest amount, hold each step for its full block, and only step up if the current dose is comfortable. Most people never need the top doses.**



STEP 4 OF 4 · DOCUMENT

# WHAT TO EXPECT + SIDE EFFECTS

Mostly mild, mostly early, mostly right after a dose increase. Know the three tiers before the first shot, not after.

## NORMAL / EXPECTED

First few days

- Reduced appetite and feeling full fast — this is tirzepatide working
- Mild nausea, burping, heartburn or reflux, especially the first 1–2 weeks after a dose increase
- Constipation or loose stools
- Redness or a small bump at the injection site; mild fatigue

## COMMON, WORTH NOTING

Manage it, don't push the dose

- Nausea was the most reported effect in the trials — it rose sharply when people started too high or stepped up too fast
- Occasional vomiting or diarrhea; headache or dizziness
- A slightly faster resting heart rate that persists during treatment
- Hair shedding or tiredness with rapid weight loss — eat enough protein

## STOP & CALL A DOCTOR

Don't push through it

- Severe, persistent stomach pain — especially spreading to the back, with or without vomiting (possible pancreatitis)
- Pain under the right ribs, fever, or yellowing skin/eyes (gallbladder)
- Vomiting that stops you keeping fluids down — dehydration risk
- A lump or swelling in the neck, hoarseness, trouble swallowing
- Signs of an allergic reaction — swelling, rash, difficulty breathing
- Shakiness, sweating or confusion if you also take insulin or blood-sugar medication (low blood sugar)

**NOTE:** for this one it is **HIGHLY RECOMMENDED** to start at the lowest amount first and work up over time, as needed. Not for anyone pregnant, breastfeeding or planning pregnancy, anyone with a personal or family history of medullary thyroid cancer or MEN 2, or a history of pancreatitis — talk to a doctor before use.



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FIRST-TIME **TL;DR**

# RECAP: CHECKLIST

Everything from pages 2–7, in the order it happens.

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## BEFORE

- ☐ Vial reconstituted with exactly 300 units of bac water, resting in the fridge, mix date written on it
  - ☐ Supplies laid out: new syringe, alcohol pad, sharps container
  - ☐ Dose set at **1.25 mg = 12.5 units** — not higher, no matter what a forum says
- 

## DURING

- ☐ Inject once per week, at night — stomach, rotate the spot
  - ☐ Note the date and dose somewhere — a phone note is fine
- 

## AFTER

- ☐ Expect possible redness, mild irritation or a headache — it settles
  - ☐ Mild nausea can happen after a dose increase — sit tight, it typically passes
  - ☐ If anything feels more than mild — dizziness, real nausea, allergic reaction — stop and get medical advice
- 

## NEXT BLOCK

- ☐ Hold each step for its full block before stepping up — never skip a step (16-week minimum cycle)
  - ☐ Only step up if the current dose is tolerated; if side effects are rough, stay another block
  - ☐ Fresh syringe, fresh pad, needle in the sharps bin — every single time
-

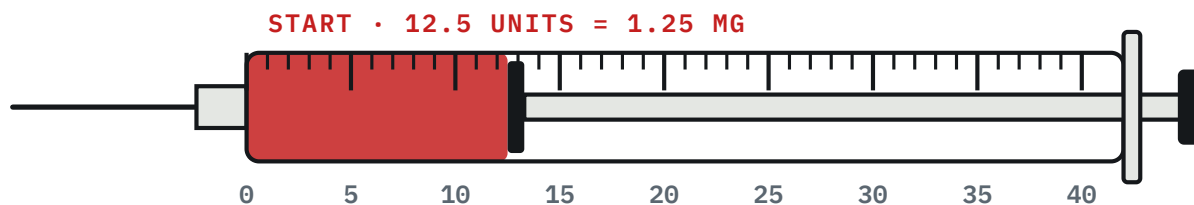


UNIT CHEAT-SHEET · 100-UNIT SYRINGE · 30 MG VIAL RECONSTITUTED WITH 300 UNITS

# HOW TO TITRATE (TITRATION SCHEDULE)

Assumes the 30 mg vial was mixed with exactly 300 units, so **1 unit = 0.1 mg**.

Units shown are per shot.



| WHEN    | DOSE                    | SYRINGE · PER SHOT   | NOTES          |
|---------|-------------------------|----------------------|----------------|
| Month 1 | <b>1.25 mg per week</b> | 12.5 units           | START HERE     |
| Month 2 | <b>2.5 mg per week</b>  | 25 units             |                |
| Month 3 | <b>5 mg per week</b>    | 50 units             |                |
| Month 4 | <b>7.5 mg per week</b>  | 75 units             | IDEAL DURATION |
| Month 5 | <b>10 mg per week</b>   | 100 units            |                |
| Month 6 | <b>12.5 mg per week</b> | 125 units – 100 + 25 |                |
| Month 7 | <b>15 mg per week</b>   | 150 units – 100 + 50 | MAX DOSE       |

**Rule of thumb:** don't move up a step until you know how the current dose feels. Only step up if the previous block was tolerated. Most people never need to go past the middle of the ramp.



## THE G2-T SOURCING GUIDE

# PROFESSOR PEPS: SOURCING GUIDE (APPENDIX)

How to verify any supplier in 90 seconds. A legitimate source clears all three checks below in under two minutes — no back-and-forth required.



1

### Ask for the COA

A batch-specific Certificate of Analysis — not a generic PDF reused across batches — showing purity  $\geq 98\%$  via HPLC/MS.

2

### Match the lot number on the vial

It must match the lot number on the COA. If they don't match, walk away.

3

### Check the track record

Searchable reviews or forum mentions older than roughly six months. Brand-new storefronts with no history are the highest-risk category.

DON'T WORRY — WE'VE ALREADY DONE THE VETTING

BioLinx Labs clears the list: third-party HPLC testing, a 99%+ purity guarantee and a COA included with every order. [biolinxlabs.com/best-sellers](https://biolinxlabs.com/best-sellers)

[biolinxlabs.com/best-sellers](https://biolinxlabs.com/best-sellers)

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## STAY CURIOUS, PROFESSOR PEPTIDES

Tirzepatide Research Hub · companion to the email series